

Alexander / Beyond-Care

Healthcare Revenue Command Layer

From denial recovery to preventable-denial intelligence, clinical documentation proof, and auditable financial truth.

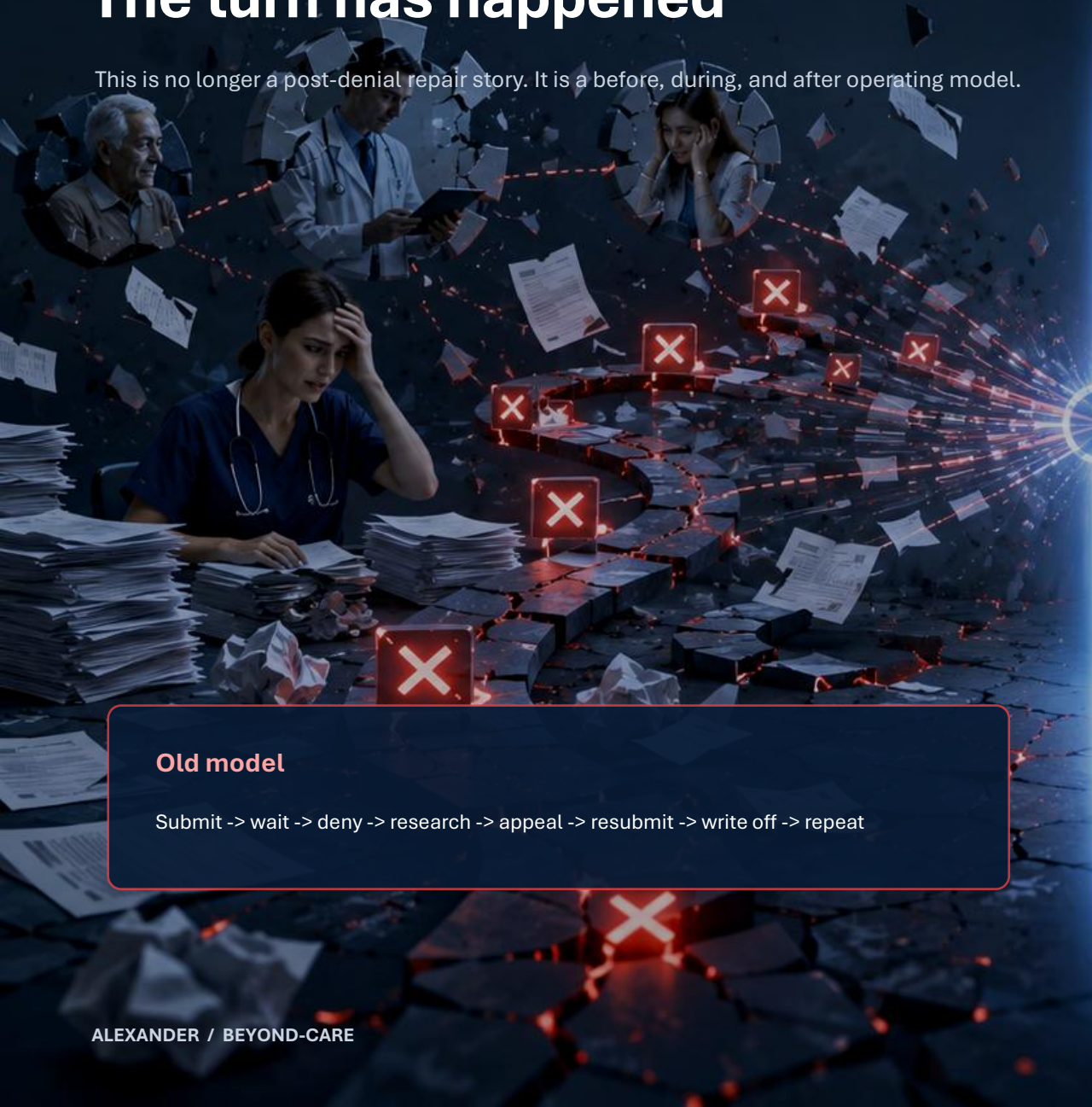
DETERMINISTIC SYSTEM + AI PRESENTATION LAYER

REPOSITORY-DRIVEN

AUDITABLE

The turn has happened

This is no longer a post-denial repair story. It is a before, during, and after operating model.



Old model

Submit -> wait -> deny -> research -> appeal -> resubmit -> write off -> repeat

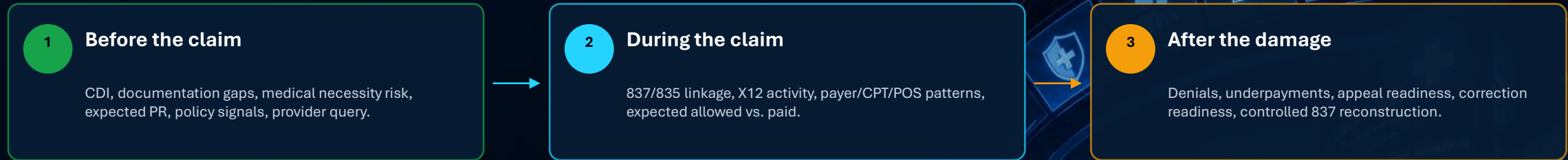


Alexander model

Analyze -> validate -> warn -> correct -> defend -> prevent

One command layer across the claim lifecycle

Alexander operates before the claim, during submission/payment, and after damage occurs.



The strategic shift: fewer avoidable denials become work queues.

The goal: reduce preventable denial exposure

Not a guarantee. A new operating discipline: catch documentation, coding, policy, and payer-risk issues before avoidable denials happen.



CDI proves the move upstream

Clinical Documentation Intelligence turns medical text into claim-prevention signals.



Before CDI, the market could pigeonhole the system as denial recovery. With CDI, the story becomes clinical-financial prevention.

The repository is the moat

Every transaction, document, rule, payment, denial, and pattern becomes reusable intelligence.

837 / 835

claims + remits

271 / 278 / X12

eligibility, auth, ops

CDI documents

notes, tests, gaps

Policy evidence

LCD/NCD/payer rules

Payment truth

allowed, paid, PR

Denial behavior

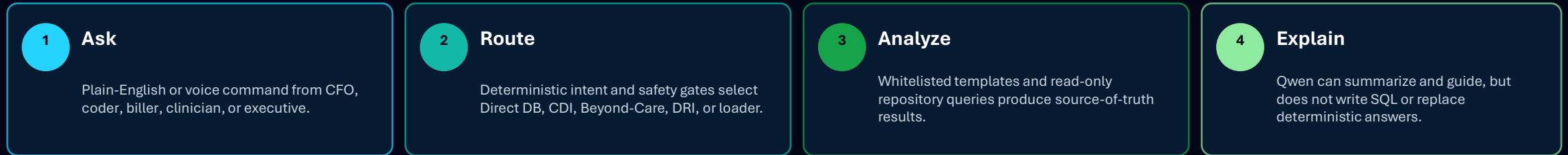
CARC/RARC/payer patterns

One source of healthcare financial truth

The value compounds because every claim event can improve future prevention, routing, proof, and recovery.

A deterministic system with an AI presentation layer

The critical distinction: AI explains and navigates. Deterministic engines decide, query, route, validate, and audit.



Ask -> Route -> Analyze -> Validate -> Reconstruct -> Appeal -> Audit

Live product proof: DB-backed CDI is already working

This is not a concept deck. The current screen loads clinical documents, extracts facts, produces code candidates, identifies gaps, and stores run summaries.

Pattern-to-Action Visual Map

User Search Options

Claims / Denials / Partial-Paid

CARC / RARC Denial Patterns

Denial Bucket Classification

Correction Readiness Grid

Reconstruction Candidate Report

Task SB — Controlled 837 Reconstruction

Task SB — Appeal Letter

Underpayment / Contract Variance

Patient Responsibility

Expected PR vs Actual PR

Provider Leakage

Care Path Economics

Clinical & Medication

Doctor Notes Intelligence

Clinical Documentation Intelligence

X12 Operations

Detail Rows

Or upload a plain-text clinical document

Drag and drop file here
Limit 200MB per file • TXT, MD, CSV

Browse files

Load pasted/uploaded CDI document

Load fake cardiology CDI demo pack

Last CDI load: document_id= | facts=31 | candidates=13 | gaps=3 | highest risk=none

Documents2

Facts31

Code Candidates13

Doc Gaps3

High Risk Gaps1

CDI Runs2

Audit boundary: CDI v1 uses deterministic phrase extraction and fixed coding/gap rules. It does not replace coder/provider review and does not call Qwen.

Documents

Extracted facts

ICD/CPT/modifier candidates

Documentation gaps

Run summaries

	document_id	batch_id	patient_id	claim_id	encounter_id	source_filename	document_type	document
0	doc_20260520035836_c1cb7ad438	batch_20260520035836_23d5940f4e	DEMO-CARD-001		ENC-c1cb7ad438	demo_cardiology_echo_supported_cdi.txt	echo_report	Demo Cai
1	doc_20260520035836_844932f892	batch_20260520035836_dff16c34b	DEMO-CARD-002		ENC-844932f892	demo_cardiology_echo_gap_cdi.txt	office_note	Demo Cai

Clinical Documentation Intelligence screen

Documents2

Facts31

Code Candidates13

Doc Gaps3

High Risk Gaps1

CDI Runs2

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Documents

Extracted facts

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Run summaries

	fact_id	document_id	patient_id	encounter_id	fact_type	body_system	fact_text
0	fact_20260520035836_4f7cc7cc4c	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	symptom	cardiology	Chief Complaint: shortness of breath and bilateral leg ede
1	fact_20260520035836_99d08d7048	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	test_result	cardiology	Chief Complaint: shortness of breath and bilateral leg ede
2	fact_20260520035836_ab86633191	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	symptom	cardiology	HPI: 68-year-old demo patient with history of hypertensic
3	fact_20260520035836_fa0694278c	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	condition	cardiology	HPI: 68-year-old demo patient with history of hypertensic
4	fact_20260520035836_b363d14574	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	history	general	HPI: 68-year-old demo patient with history of hypertensic
5	fact_20260520035836_3e21b3c1a6	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	symptom	cardiology	Exam: 2+ lower extremity edema.
6	fact_20260520035836_9922046472	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	condition	cardiology	Abnormal ECG reviewed.
7	fact_20260520035836_bbea1e640a	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	symptom	cardiology	Assessment: Heart failure symptoms, hypertension, short
8	fact_20260520035836_efb409eece	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	condition	cardiology	Assessment: Heart failure symptoms, hypertension, short
9	fact_20260520035836_93a8088f9	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	test_result	cardiology	Plan: Echocardiogram ordered to evaluate left ventricular
10	fact_20260520035836_26aaf057d2	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	order	cardiology	Plan: Echocardiogram ordered to evaluate left ventricular
11	fact_20260520035836_a0efd4689f	doc_20260520035836_c1cb7ad438	DEMO-CARD-001	ENC-c1cb7ad438	test_result	cardiology	Echocardiogram Report

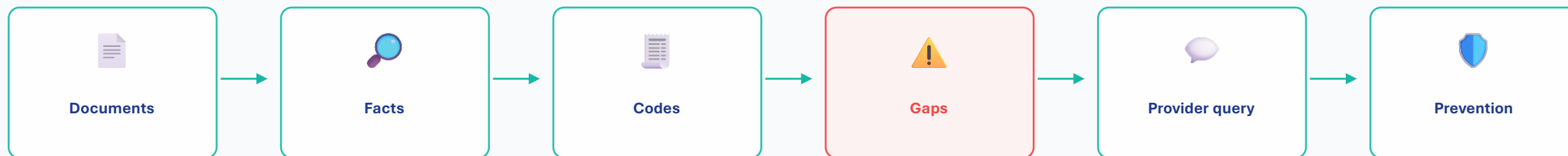
Extracted facts + code candidates

ALEXANDER / BEYOND-CARE

Deterministic system. AI presentation layer. Auditable healthcare command.

The next CDI experience should feel like a workflow map

Clickable stages, drillable documents, risk cards, provider query drafts, and proof trails - all DB-backed.

[illegible]

Competitive position: not a point solution

The market is filled with tools that each own one piece. Alexander connects the operating model.

Point-solution world

- Denial app
- Coding tool
- Eligibility check
- Appeal generator
- Analytics dashboard
- AI chat panel

Alexander world

- Clinical + financial repository
- Deterministic routing and safe DB answers
- CDI + policy + PR + X12 + denials
- Controlled correction and proof trail
- Executive command layer
- AI explanation without AI guessing

What leaders can finally ask - and answer

The deck should show executives that the system answers operating questions, not just displays reports:

What revenue is being delayed, reduced, or lost today?

Which preventable gaps are most likely to cause payer resistance?

Where are documentation, PR, policy, and contract risk converging?

Which claims can be corrected, defended, or prevented right now?

Executive answer pattern

1. One-line financial truth
2. Source tables and route audit
3. Closest proof screen
4. Recommended next action
5. Optional Qwen explanation

The message

Healthcare providers should not wait for payer denial before they understand where revenue will be delayed, reduced, or lost.

Alexander moves the operating model upstream:

documentation -> policy -> coding -> eligibility -> claim -> payment -> correction
-> audit



Alexander is the command layer above healthcare revenue chaos

Built to help providers move from reacting to denials to preventing the preventable portion of denials - with deterministic proof, auditable answers, and AI-guided navigation.

CLINICAL DOCUMENTATION INTELLIGENCE

REPOSITORY FINANCIAL TRUTH

DIRECT DB ANSWERS

AUDITABLE PROOF

Beyond Health Systems

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